

****DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 30

Age: 65 years old

Sex: Female

MRN: 000-TEST-30

Admission Date: [admission date]

Discharge Date: [discharge date]

Length of Stay: 3 days

****PRIMARY DIAGNOSIS****

Type 2 Diabetes Mellitus with hyperglycemic episode (blood glucose 487 mg/dL on admission)

****SECONDARY DIAGNOSES****

- Rheumatoid arthritis, stable on current regimen
- Gastroesophageal reflux disease (GERD), controlled
- Hypertension
- Hyperlipidemia

****HOSPITAL COURSE****

Patient 30 presented to Metro Medical Center via ED with polyuria, polydipsia, and generalized malaise × 2 days. Initial labs revealed glucose 487 mg/dL, HbA1c 9.2%, and mild metabolic acidosis (pH 7.31). Urinalysis positive for moderate ketones. Patient was admitted

to medical floor for management of hyperglycemic crisis and insulin titration.

IV fluids (0.9% NS) initiated for hydration. Endocrinology consultation obtained on hospital day 1. Basal-bolus insulin regimen was established after brief period of IV insulin drip (d/c'd on day 2 once glucose <250 mg/dL × 6 hours). Patient demonstrated good tolerability of new regimen with appropriate glucose response. Foot exam performed—no ulcers or significant neuropathy appreciated, though patient counseled on daily skin inspection. GERD and RA management continued unchanged. Patient discharged in stable condition on day 3 with glucose 156 mg/dL and improved clinical status.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
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Insulin glargine (Lantus)	20 units	q.h.s. (at bedtime)	ongoing
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Insulin lispro (Humalog)	8–12 units	with meals (AC)	ongoing; adjust per glucose log
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Metformin XR	1000 mg	BID with meals	ongoing
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Lisinopril	20 mg	q.d.	ongoing
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Atorvastatin	40 mg	q.d. at bedtime	ongoing
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Omeprazole	20 mg	BID	ongoing
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Hydroxychloroquine	200 mg	BID	ongoing
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Naproxen	500 mg	BID PRN for RA pain	ongoing
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****GLUCOSE MONITORING & DIET****

- Fingerstick glucose checks: before meals (AC) and at bedtime (QHS); additional check at 3 AM × 1 week, then reassess

- Maintain detailed glucose log for endocrinology review
- Target glucose range: 120–180 mg/dL fasting, <250 mg/dL postprandial
- Diabetic diet: consistent carbohydrate intake, avoid simple sugars; nutritionist handout provided

****ACTIVITY & RESTRICTIONS****

- No weight-bearing restrictions
- Resume normal ADLs as tolerated
- Encourage daily ambulation (30 min walking) to improve insulin sensitivity
- Avoid prolonged sitting; stand/stretch q 1 hour if desk-bound

****FOOT CARE INSTRUCTIONS****

- ✓ Inspect feet daily (top, bottom, between toes) for redness, blisters, cuts
- ✓ Wash feet in warm (not hot) water; pat dry thoroughly
- ✓ Use unscented lotion on dry skin; avoid between toes
- ✓ Wear well-fitting, supportive shoes